Wuckert Group Regional Medical Center

8326 Church St, Newark, NJ 07112

CONSOLIDATED PATIENT RECORDS — CONFIDENTIAL

Report Generated: 04/13/2026 05:57 PM
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PATIENT 1 OF 6

Name: Jodie Johnson
DOB: 05/13/1951
Address: 6531 Broad St, Savannah, GA 31404
Insurance: INS-9256578530
Admitted: 04/11/2026

MRN: MRN-46212599
SSN: 570-76-3593
Phone: (843) 408-7458
Attending: Dr. Pablo Jones
Discharged: 04/20/2026

VITALS

BP: 160/61 mmHg HR: 87 bpm Temp: 98.9 F SpO2: 93% RR: 20 breaths/min

DIAGNOSES

- Osteoarthritis, unspecified (M19.90)
- Gastroesophageal Reflux Disease (K21.0)
- Major Depressive Disorder, recurrent (F33.0)
- Hyperlipidemia (E78.5)

MEDICATIONS

- Omeprazole 20mg — once daily before breakfast
- Metoprolol 25mg — twice daily
- Amlodipine 5mg — once daily
- Levothyroxine 75mcg — once daily on empty stomach
- Atorvastatin 20mg — once daily at bedtime
- Lisinopril 10mg — once daily
- Prednisone 10mg — taper over 7 days

HOSPITAL COURSE

Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
The patient's vital signs remained stable throughout the shift with no acute changes noted on continuous monitoring.
Foley catheter care performed; output adequate and within expected range; plan to discontinue catheter per protocol tomorrow.
Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
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Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.

Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
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SUBSTANCE ABUSE & ADDICTION ASSESSMENT
Patient: Jodie Johnson SSN: 570-76-3593 MRN: MRN-46212599
Methadone maintenance therapy continued at 80mg daily during hospitalization per outpatient opioid treatment program.
AUDIT-C score of 8 indicates hazardous drinking pattern; brief intervention performed with motivational interviewing techniques.
Buprenorphine/naloxone (Suboxone) 8mg/2mg sublingual film initiated for opioid use disorder during this admission.
Naloxone rescue kit prescribed at discharge given history of opioid use disorder; patient and family educated on administration.
Assessed by: Dr. Stephon Collier, Addiction Medicine
42 CFR Part 2 protected information — unauthorized disclosure prohibited.
— End of record for Jodie Johnson (MRN: MRN-46212599) —

PATIENT 2 OF 6

Name: Hulda Walsh
DOB: 10/08/1940
Address: 2801 Washington Blvd, Pittsburgh, PA 15219
Insurance: INS-2440392072
Admitted: 04/11/2026

MRN: MRN-90657341
SSN: 769-87-5649
Phone: (912) 722-4749
Attending: Dr. Adell Runte
Discharged: 04/19/2026

VITALS
BP: 106/91 mmHg HR: 92 bpm Temp: 100.4 F SpO2: 93% RR: 15 breaths/min

DIAGNOSES
- Hyperlipidemia (E78.5)
- Essential Hypertension (I10)
- Urinary Tract Infection (N39.0)
- Osteoarthritis, unspecified (M19.90)
- Chronic Obstructive Pulmonary Disease (J44.1)

MEDICATIONS
- Metoprolol 25mg — twice daily
- Gabapentin 300mg — three times daily
- Prednisone 10mg — taper over 7 days
- Omeprazole 20mg — once daily before breakfast
- Amlodipine 5mg — once daily
- Levothyroxine 75mcg — once daily on empty stomach

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PSYCHIATRIC EVALUATION & MENTAL HEALTH NOTES

Patient: Hulda Walsh DOB: 10/08/1940 SSN: 769-87-5649
Patient endorsed auditory hallucinations described as commanding voices; antipsychotic medication adjusted accordingly.
Patient carries an active diagnosis of Bipolar I Disorder with psychotic features (F31.2); lithium levels monitored during admission.
Patient diagnosed with Posttraumatic Stress Disorder (F43.10) secondary to military combat exposure; nightmares and hypervigilance reported.
Eating disorder history documented: anorexia nervosa, restricting type, with BMI of 16.2 on admission; nutrition consult placed.
History of three prior psychiatric hospitalizations for acute mania with grandiose delusions and decreased need for sleep.
Evaluating Psychiatrist: Dr. Erna Steuber, MD, Board Certified Psychiatry
Protected mental health information per state and federal law.

— End of record for Hulda Walsh (MRN: MRN-90657341) —

PATIENT 3 OF 6

Name: Deanna Berge
DOB: 04/08/1974
Address: 445 Broad St, Charlotte, NC 28204
Insurance: INS-1677005623
Admitted: 04/11/2026

MRN: MRN-74112839
SSN: 520-96-3683
Phone: (770) 787-1562
Attending: Dr. Lavern Gibson
Discharged: 04/18/2026

VITALS

BP: 100/73 mmHg HR: 67 bpm Temp: 99.6 F SpO2: 99% RR: 17 breaths/min

DIAGNOSES

- Atrial Fibrillation (I48.91)
- Chronic Obstructive Pulmonary Disease (J44.1)
- Acute Upper Respiratory Infection (J06.9)

MEDICATIONS

- Omeprazole 20mg — once daily before breakfast
- Amoxicillin 500mg — three times daily for 10 days
- Atorvastatin 20mg — once daily at bedtime
- Amlodipine 5mg — once daily
- Metformin 500mg — twice daily
- Sertraline 50mg — once daily
- Prednisone 10mg — taper over 7 days

HOSPITAL COURSE

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HIV/STI SCREENING & TREATMENT NOTES

Patient: Deanna Berge MRN: MRN-74112839 Insurance: INS-1677005623
CD4 count obtained during admission: 485 cells/mm3; viral load undetectable (<20 copies/mL) confirming virologic suppression.
HIV genotype resistance testing performed; no major resistance mutations identified on current regimen.
Hepatitis B surface antigen positive; HBV DNA viral load of 45,000 IU/mL; hepatology referral for treatment initiation.
RPR reactive with titer of 1:64; confirmatory FTA-ABS positive consistent with secondary syphilis; IM penicillin G administered.
Infectious Disease Consult: Dr. Diana Champlin
HIV-related information protected under state confidentiality statutes.

— End of record for Deanna Berge (MRN: MRN-74112839) —

PATIENT 4 OF 6

Name: May Wunsch
DOB: 04/22/1936
Address: 8300 Oak Ave, Washington, DC 20010
Insurance: INS-7133649096
Admitted: 04/11/2026

MRN: MRN-15124379
SSN: 903-29-6815
Phone: (804) 624-8938
Attending: Dr. Luther Murazik
Discharged: 04/13/2026

VITALS

BP: 111/71 mmHg HR: 61 bpm Temp: 97.0 F SpO2: 100% RR: 13 breaths/min

DIAGNOSES

- Gastroesophageal Reflux Disease (K21.0)
- Osteoarthritis, unspecified (M19.90)
- Urinary Tract Infection (N39.0)

MEDICATIONS

- Amlodipine 5mg — once daily
- Amoxicillin 500mg — three times daily for 10 days
- Lisinopril 10mg — once daily
- Prednisone 10mg — taper over 7 days
- Levothyroxine 75mcg — once daily on empty stomach

HOSPITAL COURSE

Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
Patient education provided regarding the importance of ambulation and incentive spirometry use to prevent postoperative complications.
Foley catheter care performed; output adequate and within expected range; plan to discontinue catheter per protocol tomorrow.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as

ordered.
Nursing assessment performed at the bedside; patient is resting comfortably with no complaints of pain or distress.
Occupational therapy assessed patient's ability to perform activities of daily living; mild difficulty with fine motor tasks noted.
Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
Medication reconciliation reviewed with patient and pharmacy; no discrepancies identified between home and inpatient medication lists.
Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Blood glucose monitoring performed per sliding scale protocol; values within target range and insulin administered as ordered.
Physical therapy facilitated ambulation in the hallway with rolling walker; patient ambulated 150 feet with minimal assistance.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
Sequential compression devices applied bilaterally for DVT prophylaxis; patient instructed to keep devices on while in bed.
Respiratory therapy performed scheduled nebulizer treatment; patient tolerated well with improved breath sounds bilaterally.
Foley catheter care performed; output adequate and within expected range; plan to discontinue catheter per protocol tomorrow.
Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.
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Laboratory specimens collected per physician orders and sent to the lab; results anticipated within standard turnaround time.

Dietary tray delivered; patient consumed approximately 75% of meal with good appetite noted by nursing staff.
Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
Intravenous fluids continued at maintenance rate; intake and output documented and within acceptable parameters.
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Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.

GENETIC TESTING RESULTS & COUNSELING NOTES

Patient: May Wunsch DOB: 04/22/1936 SSN: 903-29-6815
Whole exome sequencing identified pathogenic variant in CFTR gene consistent with cystic fibrosis carrier status.
Pharmacogenomic testing revealed CYP2D6 poor metabolizer status; codeine and tramadol contraindicated per FDA labeling.
Hereditary hemochromatosis: HFE C282Y homozygous genotype confirmed; therapeutic phlebotomy schedule initiated.
Genetic Counselor: Shannon Johnston, MS, CGC
GINA-protected genetic information — disclosure restrictions apply.

— End of record for May Wunsch (MRN: MRN-15124379) —

PATIENT 5 OF 6

Name: Arnold Marquardt
DOB: 06/09/1958
Address: 637 Maple Dr, Charleston, SC 29403
Insurance: INS-5743921281
Admitted: 04/11/2026

MRN: MRN-55376099
SSN: 002-30-4618
Phone: (904) 920-3677
Attending: Dr. Leopold MacGyver
Discharged: 04/18/2026

VITALS

BP: 105/86 mmHg HR: 91 bpm Temp: 97.5 F SpO2: 94% RR: 23 breaths/min

DIAGNOSES

- Osteoarthritis, unspecified (M19.90)
- Acute Upper Respiratory Infection (J06.9)
- Anxiety Disorder, unspecified (F41.9)
- Essential Hypertension (I10)

MEDICATIONS

- Amoxicillin 500mg — three times daily for 10 days
- Lisinopril 10mg — once daily

- Metformin 500mg — twice daily
- Prednisone 10mg — taper over 7 days

HOSPITAL COURSE

Patient participated in bedside physical therapy exercises including ankle pumps, quad sets, and straight leg raises.
Pain management reassessed per scheduled protocol; patient reports pain level of 3/10 with current medication regimen.
Patient's family visited and was updated on the plan of care by the attending physician during afternoon rounds.
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DOMESTIC VIOLENCE / INTIMATE PARTNER VIOLENCE SCREENING

Patient: Arnold Marquardt Address: 637 Maple Dr, Charleston, SC 29403 Phone: (904) 920-3677
Patient provided with National Domestic Violence Hotline number (1-800-799-7233) and local advocacy resources.
Intimate partner violence screening positive; patient reports physical abuse by current partner occurring 2-3 times monthly.
Mandatory reporting completed per state law given suspected abuse involving a vulnerable adult; Adult Protective Services notified.
Screened by: River Schinner, LCSW
Sensitive domestic violence documentation — restricted access per hospital policy.

— End of record for Arnold Marquardt (MRN: MRN-55376099) —

PATIENT 6 OF 6

Name: Waino Terry
DOB: 04/23/1977
Address: 2234 Commonwealth Ave, Baltimore, MD 21231
Insurance: INS-8140979101

MRN: MRN-82638426
SSN: 834-12-3095
Phone: (959) 272-1610
Attending: Dr. Arden Rohan

Admitted: 04/11/2026 Discharged: 04/18/2026

VITALS

BP: 128/83 mmHg HR: 75 bpm Temp: 101.1 F SpO2: 93% RR: 17 breaths/min

DIAGNOSES

- Atrial Fibrillation (I48.91)
- Hyperlipidemia (E78.5)
- Osteoarthritis, unspecified (M19.90)

MEDICATIONS

- Amoxicillin 500mg — three times daily for 10 days
- Omeprazole 20mg — once daily before breakfast
- Prednisone 10mg — taper over 7 days
- Atorvastatin 20mg — once daily at bedtime
- Metoprolol 25mg — twice daily
- Sertraline 50mg — once daily

HOSPITAL COURSE

Wound care performed per protocol with sterile technique; wound bed appears clean and granulating without signs of infection.
Fall risk assessment completed using the Morse Fall Scale; patient scored moderate risk and appropriate precautions implemented.
The patient's vital signs remained stable throughout the shift with no acute changes noted on continuous monitoring.
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Interdisciplinary team rounds completed with input from medicine, nursing, pharmacy, physical therapy, and case management.

SUBSTANCE ABUSE & ADDICTION ASSESSMENT

Patient: Waino Terry SSN: 834-12-3095 MRN: MRN-82638426
Patient admitted to daily cannabis use for the past 3 years; counseled on cessation and referred to addiction services.
CAGE questionnaire score of 3/4; patient acknowledges problematic alcohol consumption averaging 12-15 drinks per week.
Patient has a documented history of alcohol use disorder, moderate severity, currently in early remission per DSM-5 criteria (F10.20).
Methadone maintenance therapy continued at 80mg daily during hospitalization per outpatient opioid treatment program.

AUDIT-C score of 8 indicates hazardous drinking pattern; brief intervention performed with motivational interviewing techniques.
Naloxone rescue kit prescribed at discharge given history of opioid use disorder; patient and family educated on administration.
Assessed by: Dr. Orin Vandervort, Addiction Medicine
42 CFR Part 2 protected information — unauthorized disclosure prohibited.
— End of record for Waino Terry (MRN: MRN-82638426) —

END OF CONSOLIDATED PATIENT RECORDS
Total patients: 6 | Generated: 04/13/2026